

TRANSFORMING AMERICAN HEALTHCARE

A Six-Pillar Framework for System Transformation

Vermont as the National Template | April 2026

Health Transformation Review | htreview.com

PART I

THE FOUNDATION

Framework, Portrait, and Reader's Guide

CHAPTER 1

The Six-Pillar Framework

THE SIX-PILLAR TRANSFORMATION FRAMEWORK

1 POLICY

Mandatory architecture
for structural reform

2 ECONOMICS

Payment reform &
global budget design

3 TECHNOLOGY

Data infrastructure
for population health

4 CLINICAL

Care delivery redesign
for the transformed system

5 EQUITY

Closing gaps, not
just averaging them

6 OPERATIONS

Where strategy
becomes execution

All six must move together — single-pillar interventions consistently fail

WHY SINGLE-PILLAR INTERVENTIONS FAIL

SINGLE-PILLAR APPROACH → FAILS

Payment Reform Only

Hospitals have no population health infrastructure to respond

Clinical Redesign Only

Payment system penalizes utilization reduction with lost revenue

Technology Only

Platforms deployed with no incentive or capacity to use them

Equity Program Only

Structural drivers — access, payment, SDOH — go unaddressed

SIX-PILLAR APPROACH → WORKS

Policy enables Economics

Mandatory RBP/global budgets remove voluntary opt-out by highest-cost actors

Economics drives Clinical

Payment incentives align with population health model requirements

Technology enables all

Data infrastructure makes population health management scalable

Equity monitors all

Every pillar decision evaluated for equity implications — no blind spots

VERMONT'S SIX-PILLAR TRANSFORMATION MAP

Pillar	Primary Vermont Intervention	Act / Vehicle	Key Deadline
Policy	Acts 167 (2022), 51, 55, 68 (2025); AHEAD State Agreement	Vermont legislature + CMS	December 2028 Strategic Plan
Economics	Mandatory RBP (FY2027); Hospital global budgets (FY2028-2030); AHEAD TCOC	GMCB + DVHA	FY2027 / FY2028 / FY2030
Technology	VHCURES all-payer database; Vermont CIN; AHS-GMCB analytics vendor	GMCB + DVHA + AHS	Analytics vendor 2026
Clinical	Blueprint for Health (all-payer PCMH+CHT); CoCM; CCBHC expansion (7 entities)	DVHA + Blueprint + DMH	5 new CCBHCs by July 2026
Equity	Geographic equity monitoring; SDOH integration; GLP-1 BALANCE decision	AHS + VDH + DVHA	BALANCE decision 2026
Operations	14-hospital transformation plans; Vermont CIN shared services; PMO	AHS + 14 hospitals	Final plans early 2026

VERMONT'S LEGISLATIVE REFORM CASCADE 2022 – 2025

2022 Act 167	2023 Act 51	2025 (Spring) Act 55	2025 (Summer) Act 68
The Diagnosis Mandate Hospital systems analysis commissioned. Oliver Wyman engaged. Five goals established: reduce inefficiencies, lower costs, improve access, improve outcomes, reduce inequities.	The Foundation Hospital sustainability assessments. CON process review. Framework for subsequent payment reform legislation.	Drug Pricing Cap \$135M in annual drug savings. Hospital 340B reporting requirements. Act 55 drug cap: \$104.3M in FY2026 savings.	The Mandate Mandatory RBP (FY2027). Global budgets (FY2028-2030). Statewide Strategic Plan (December 2028). Health Care Delivery Advisory Committee.

CHAPTER 2

The Vermont System Portrait

VERMONT: THE DEMOGRAPHIC CRISIS IN DATA

647K

Total population

2nd smallest US state by population

57%

Growth in 65+ population by 2040

From 21.7% today to 30%+ of total

-13%

Working-age population by 2040

From 367K to ~318K — shrinking commercial base

97%

Insured rate — one of the highest in US

ACA + Medicaid expansion; yet still unaffordable

108%

Premium increase 2018–2024

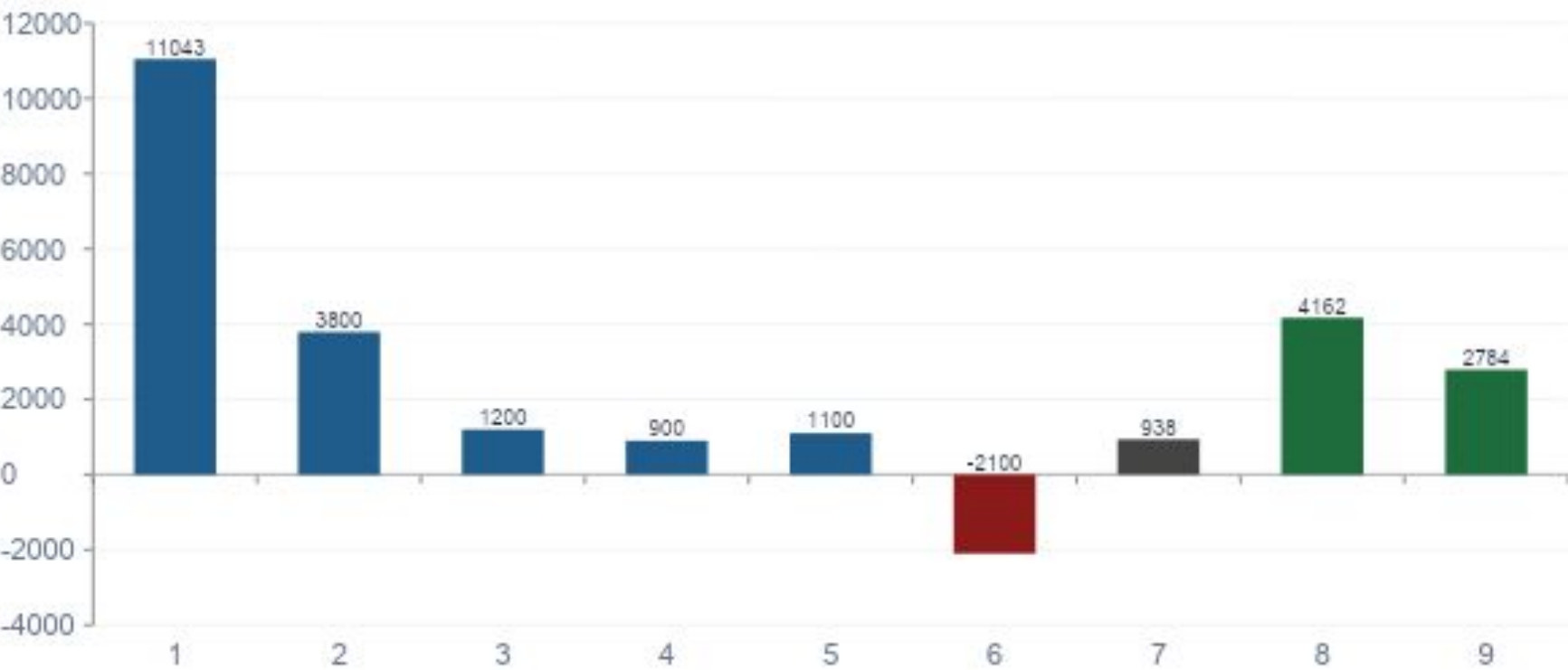
\$456/month → \$948/month; income grew only 22%

9/14

Hospitals reporting losses (FY2023)

Worst performing: -8.9% operating margin

VERMONT HOSPITAL FINANCIAL POSITIONS: FY2023 vs. BENCHMARKS



VERMONT'S 14-HOSPITAL NETWORK: THREE-TIER STRUCTURE

TIER 1

Regional Specialty Centers (RSC)

UVMMC (16 COEs) | SW Vermont Medical | Brattleboro Memorial | Rutland Regional | Central VT Medical

Highest COE count. Accept referrals from the network for designated specialties. Financial sustainability strongest. Primary RSC role: UVMMC.

TIER 2

Community Hospitals — Focused Scope

NW Medical Center | NE VT Regional | Springfield | Copley | Mt. Ascutney | Porter Medical

2–4 COE designations. Maintain specific service lines. Transfer complex cases to RSCs. Actively engaged in Act 68 transformation planning.

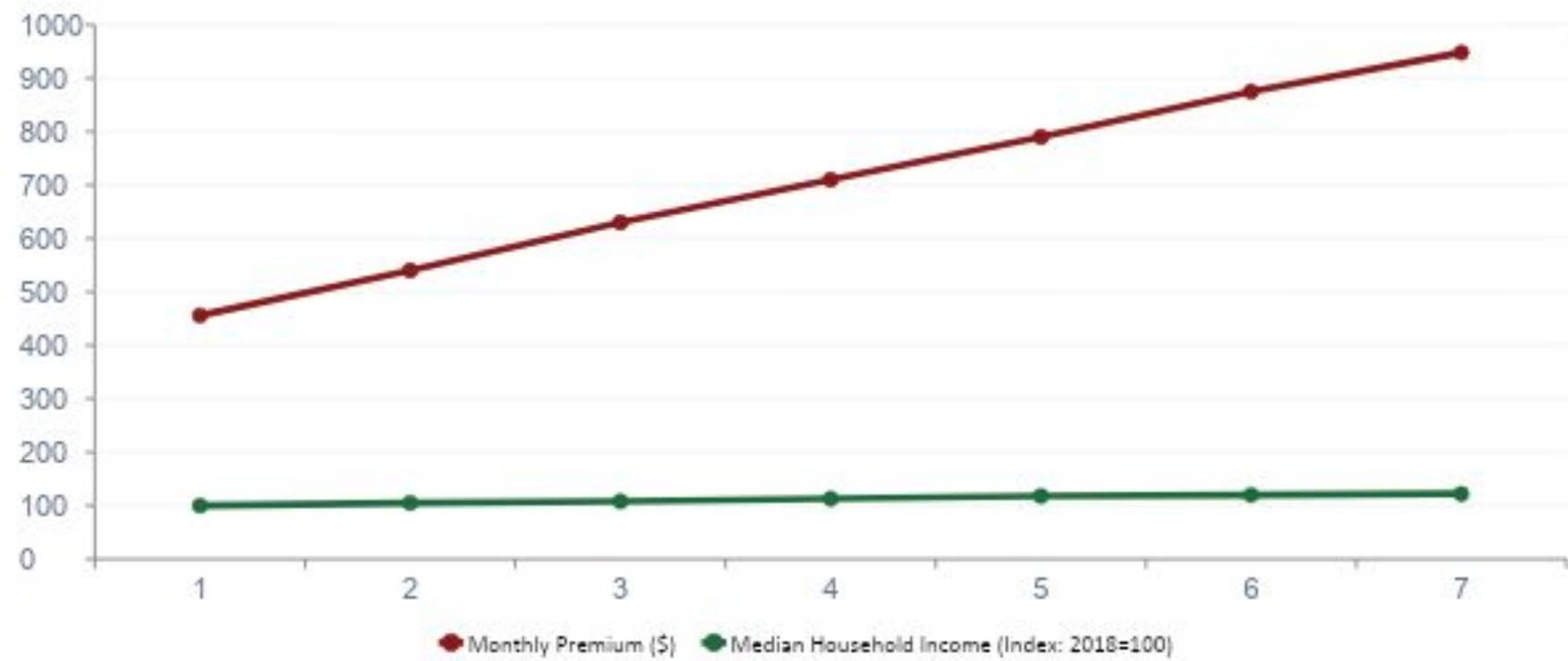
TIER 3

Transition Assessment Required

Grace Cottage (19 beds) | Gifford Medical | North Country Hospital

Most financially vulnerable. REH conversion, CACC, or home-based care model under assessment. Act 68 transformation plans address transition path.

VERMONT PREMIUM vs. INCOME GROWTH 2018–2024: THE AFFORDABILITY GAP



108% premium increase vs. 22% income growth — the affordability crisis driving Vermont's transformation mandate

PART II

THE SIX PILLARS IN DEPTH

Policy · Economics · Operations · Clinical · Equity · Technology

CHAPTER 4

The Policy Pillar — Legislative Architecture

OLIVER WYMAN'S THREE IMPERATIVES FOR VERMONT

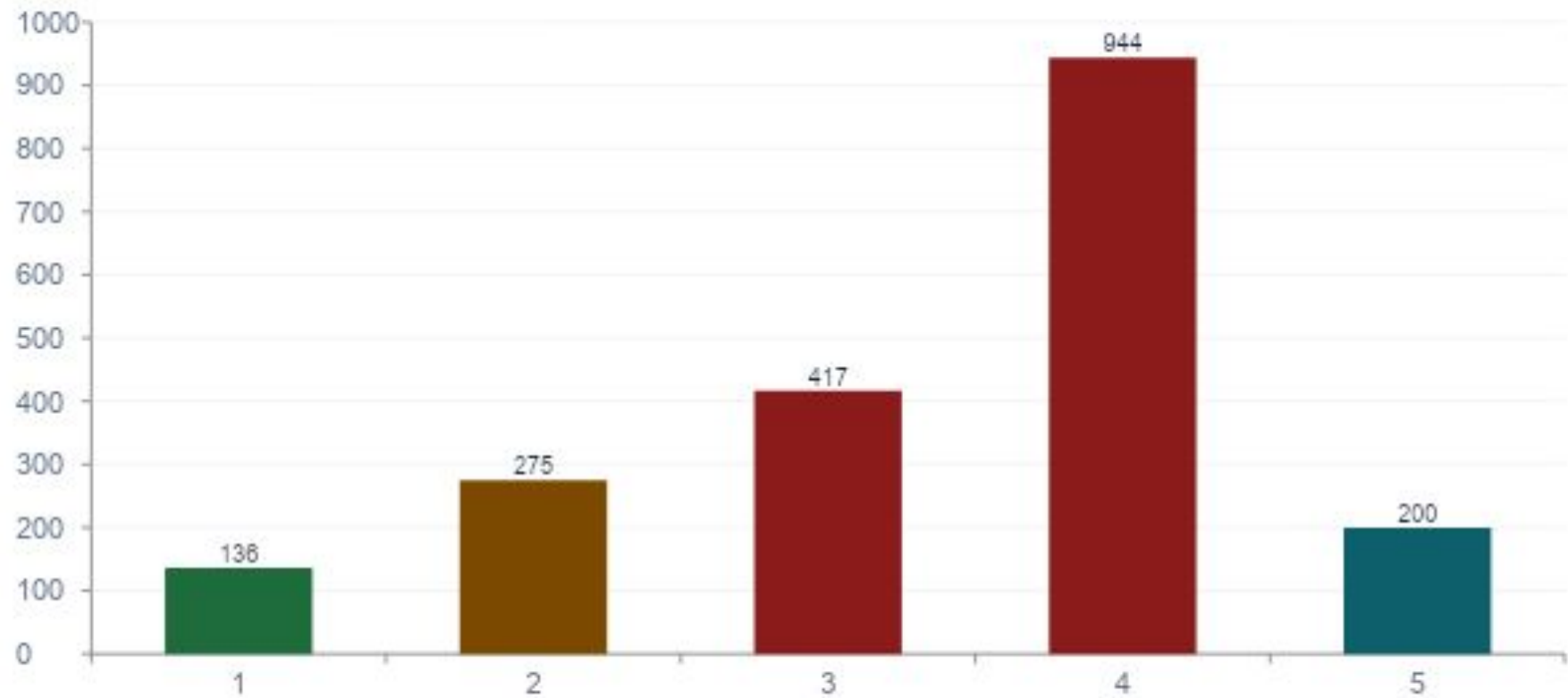
230 community meetings · 14 hospital service areas · \$1M analysis commissioned under Act 167

IMPERATIVE 1	IMPERATIVE 2	IMPERATIVE 3
BUILD HOUSING AND FIX TRANSPORTATION WHY: Housing vacancy rate 3% (healthy = 5%). Half of renters cost-burdened. Provider recruitment impossible without housing. Transportation barriers = missed appointments = crisis presentations. WHAT: Advocacy for housing investment as healthcare policy. Transportation as a health equity intervention. Oliver Wyman's FIRST imperative — before payment reform.	PAY WITH RBP AND MOVE TO GLOBAL BUDGETS WHY: Commercial prices average 250-300% of Medicare. UVMHC outpatient: 417% of Medicare. Premium increased 108% in 6 years. Break-even: ~136% of Medicare. WHAT: Act 68 mandates RBP (FY2027) and hospital global budgets (FY2028-2030). AHEAD Model provides Medicare FFS global budget from January 2027.	MOVE CARE OUT OF HOSPITALS WHY: 32.3% of ED visits potentially avoidable. Hospital-centric model is financially unsustainable as demographics shift cost toward Medicare/Medicaid. Administrative overhead 91% above benchmark. WHAT: Blueprint PCMH expansion. CCBHC network. CoCM behavioral health integration. RPM and telehealth for chronic disease management.

ACT 68 OF 2025 — KEY PROVISIONS AND DEADLINES

Provision	Requirement	Deadline	Authority
FY2026 rate reduction	–1% commercial rate benchmark; \$230.65M total regulatory reductions	FY2026 (ongoing)	GMCB
Transformation grants	\$2M to hospitals for transformation plan development and implementation	September 2025 launch	AHS
Reference-Based Pricing	Mandatory maximum commercial rates for all Vermont hospitals	FY2027 (Oct 2026)	GMCB
Medicare global budgets	AHEAD Model Cohort 2; Medicare FFS global budgets for all hospitals	January 1, 2027	AHS/CMS
Non-CAH global budgets	Prospective annual budgets for PPS hospitals	FY2028 (Oct 2027)	GMCB
All-hospital global budgets	CAHs included in mandatory global budget program	FY2030 (Oct 2029)	GMCB
Statewide Strategic Plan	Delivered to Legislature; updated every 3 years	December 1, 2028	AHS + Advisory Committee

VERMONT HOSPITAL COMMERCIAL PRICES vs. MEDICARE (% of Medicare)



Act 68 RBP will set mandatory commercial maximum rates — ending the pricing that drives Vermont's 108% premium increase
Chapter 4 / Chapter 6 — Source: GNCB Act 68 RBP Update (February 2026); RAND Hospital Transparency; GNCB FY2026 data

THE AHEAD MODEL — VERMONT'S FEDERAL PARTNERSHIP (COHORT 2)

Hospital Global Budgets

All Vermont hospitals receive prospective Medicare FFS global budgets from Jan 2027. Hospitals receive fixed revenue regardless of volume — incentive shifts to population health management.

Primary Care AHEAD

Voluntary enhanced Medicare payments for PCMHs. Four payment pathways including fully prospective models. Replaces expiring Blueprint Medicare PMPM demonstration.

EAST Fund

Up to \$150M/year in additional Medicare funds for Vermont to invest in primary care, behavioral health, home health, long-term care, and SDOH programs — the transformation capital engine.

Total Cost of Care Accountability

Vermont responsible for all-payer TCOC targets. State-level accountability with meaningful consequences after Year 3. 9-year model: January 2027 through December 2035.

KEY AGREEMENT TERMS: Signed January 17, 2025 · Vermont: Cohort 2 · Starts January 1, 2027 · 9-year model · Vermont may withdraw with 30-day notice (pre-launch) or 6-month notice (post-launch)

CHAPTER 5

The Policy Pillar in Practice — CMMI, Waivers, H.R.1

H.R. 1 — THE ONE BIG BEAUTIFUL BILL ACT: HEALTHCARE IMPACT

\$911B

Federal Medicaid spending cuts
over 10 years (CBO estimate)

10M

Additional uninsured Americans
by 2034 (CBO)

\$137B

Rural Medicaid spending reduction
over 10 years (KFF)

\$50B

Rural Health Transformation
Program (5 years, 2026-2030)

THE FISCAL CLIFF PROBLEM

The RHT Program is FRONT-LOADED (2026-2030) while Medicaid cuts are BACK-LOADED — 64% of 10-year Medicaid reductions occur AFTER 2030 when the RHT fund expires. Vermont must use RHT capital to build structural efficiency by 2030, or face the Medicaid revenue cliff without the cost structure to absorb it.

CMMI MODEL SUCCESS FACTORS: WHY MOST FAIL AND SOME WORK

Factor	Successful Models	Failed Models
Mandatory participation	AHEAD, Maryland HSCRC, Vermont Act 68 — all providers must participate	Most CMMI ACOs, BPCI-A, bundled payments — highest-cost actors opt out
Two-sided risk (real)	ACO REACH full risk, AHEAD global budgets — meaningful downside	MSSP Track 1 (upside only), many bundles with minimal downside exposure
State regulatory alignment	AHEAD (designed with state agencies), Maryland HSCRC (built on existing authority)	Federal models imposed without coordinating with state Medicaid, insurance regulators
Implementation support	AHEAD cooperative agreement funds, Vermont RHRC engagement, pre-launch period	Models launched with insufficient runway — providers cannot redesign care before accountability begins

SECTION 1115 WAIVERS — THE STATE'S PRIMARY FEDERAL POLICY TOOL

Coverage Expansions

National: Extending Medicaid to populations not otherwise eligible; non-traditional benefit structures

Vermont: Vermont Global Commitment to Health: expanded coverage beyond standard state plan requirements

Payment Reform

National: Global budgets, ACO arrangements, APMs for Medicaid not permitted under standard rules

Vermont: Vermont's Medicaid hospital global budgets under AHEAD alignment operate via Global Commitment demonstration

SDOH Investments

National: Housing supports, food assistance, transportation for high-risk Medicaid enrollees

Vermont: Vermont HRSN amendment authorizes Medicaid-funded social services connected to Blueprint HRSN screening

Budget Neutrality Constraint

National: Federal spending cannot exceed what it would have been without the waiver — negotiated baseline

Vermont: AHEAD methodology for Vermont's Medicaid global budget baseline is a critical negotiated parameter with major financial consequences



“The RHT funding is a tool that lets us put affordability plans into action. This is not our health care reform plan. This is an opportunity that we’ll use to support all of the health care reform work that we’re already doing in the state.”

— Jenney Samuelson, Vermont AHS Secretary | January 2026

CHAPTER 6

The Economics Pillar — Global Budgets & RBP

FEE-FOR-SERVICE vs. GLOBAL BUDGET: THE INCENTIVE REVOLUTION

FEE-FOR-SERVICE

Revenue per discharge — MAXIMIZE

Admission volume — MORE = MORE REVENUE

Length of stay — LONGER = MORE REVENUE

Service line volume — GROW PROFITABLE LINES

Payer mix — MORE COMMERCIAL = BETTER

Denials management — RECOVER DENIED CLAIMS

Case mix index — CAPTURE HIGHER ACUITY

GLOBAL BUDGET

Cost per discharge — MINIMIZE UNNECESSARY COST

Avoidable admission rate — LOWER IS BETTER

Efficient LOS — UNNECESSARY DAYS = COST

PMPM population health cost — STAY IN BUDGET

Quality performance — DETERMINES ADJUSTMENTS

Potentially avoidable utilization — PREVENT THEM

Community investment ROI — SDOH PROGRAMS SAVE MONEY

MARYLAND'S GLOBAL BUDGET PROOF OF CONCEPT

Maryland Health Services Cost Review Commission — results after 10+ years of all-payer global hospital budgets

\$1.4B

Medicare savings generated

Cumulative savings vs. national trend

-7%

Hospital admission rate reduction

Compared to national average trajectory

30yr

Duration of global budget model

Longest-running all-payer hospital budget system

#1

Medicare per-capita spending rank

Maryland moved from high-cost to lowest-cost state

WHAT MARYLAND PROVES FOR VERMONT

Global budgets can produce documented Medicare savings and utilization reduction. The model requires: (1) Mandatory participation — no opt-out. (2) All-payer design — Medicare, Medicaid, and commercial aligned. (3) State regulatory authority — Maryland's HSCRC has 30 years of institutional capacity. Vermont's GMCB is building this capacity now.

VERMONT'S TOTAL COST OF CARE ACCOUNTABILITY ARCHITECTURE

FEDERAL LAYER

AHEAD Model TCOC Target

CMS sets state-level total cost of care benchmark for Vermont Medicare FFS. Vermont accountable for staying below benchmark. Consequences after Year 3. Up to \$150M/year EAST Fund.

STATE LAYER

GMCB Global Hospital Budgets

Each Vermont hospital receives a prospective annual budget for Medicare FFS. Fixed revenue regardless of volume. Incentive: efficient population health management. Mandatory from FY2028.

PRIMARY CARE LAYER

Primary Care AHEAD Enhanced Payments

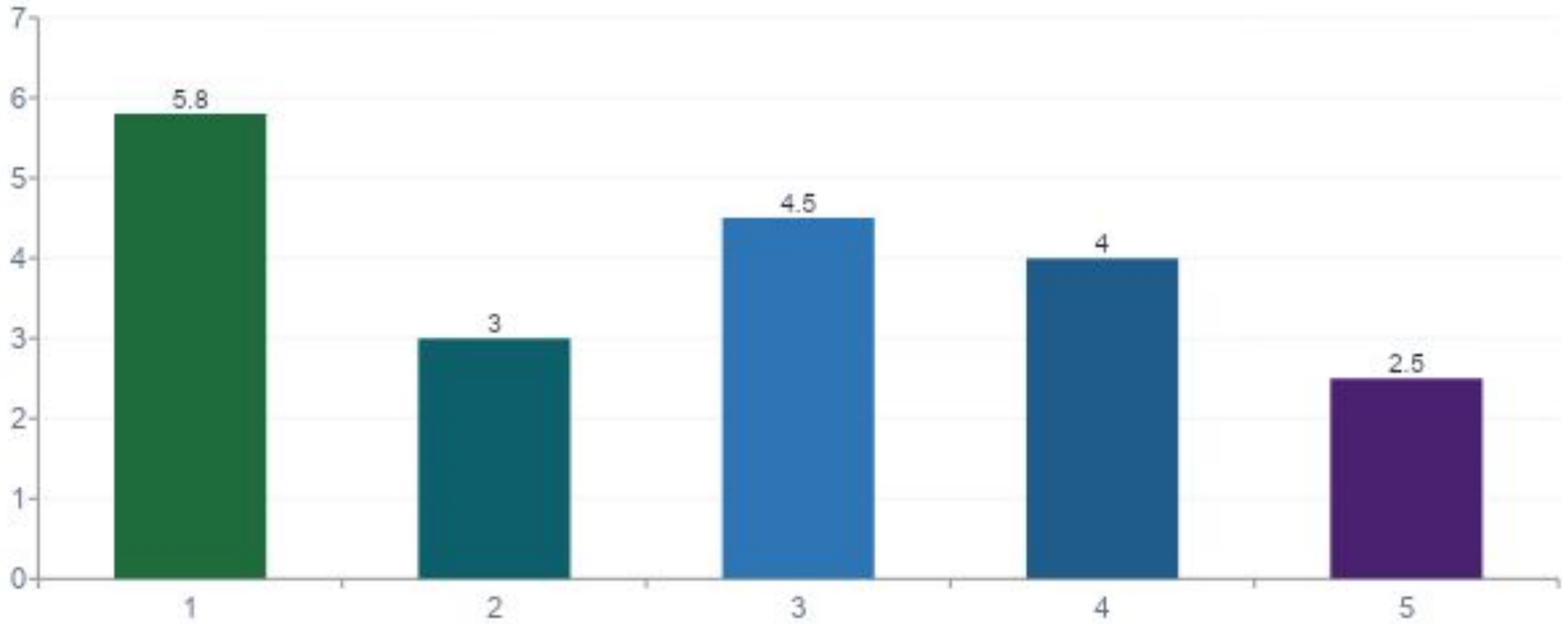
Vermont PCMH practices receive enhanced prospective Medicare payments under PC AHEAD pathways. Replaces expiring Blueprint Medicare PMPM demonstration. Voluntary — encourages expansion.

INVESTMENT LAYER

EAST Fund — Transformation Capital

Vermont receives up to \$150M/year in additional Medicare funds for primary care, behavioral health, SDOH, home health, long-term care investments that the payment reform requires.

THE FINANCIAL RETURN ON PREVENTION UNDER GLOBAL BUDGETS



Under FFS, prevention costs money. Under global budgets, prevention saves money. Vermont's Blueprint proves the ROI — \$5.8M reduction in medical expenditure per \$1M invested (Population Health Management journal).

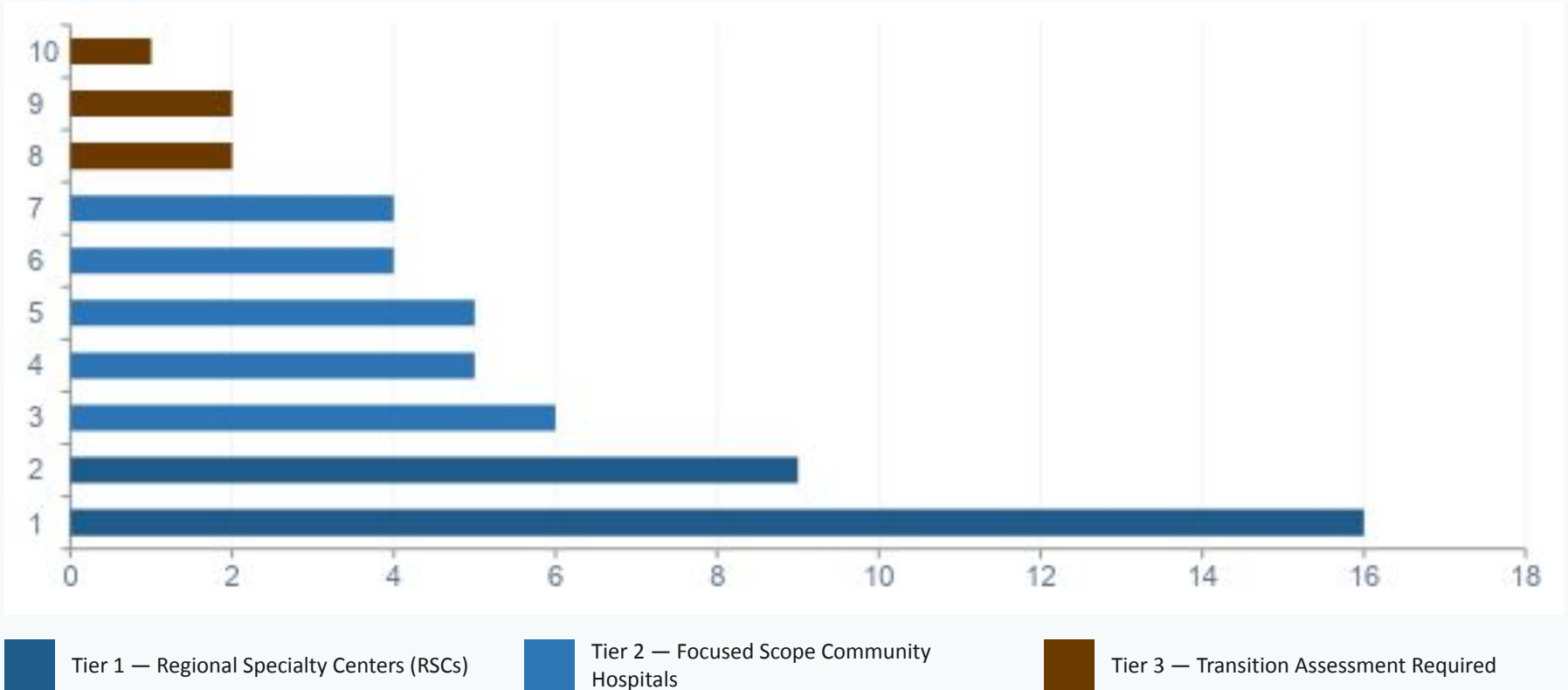
CHAPTER 8

The Operations Pillar — Hospital System Transformation

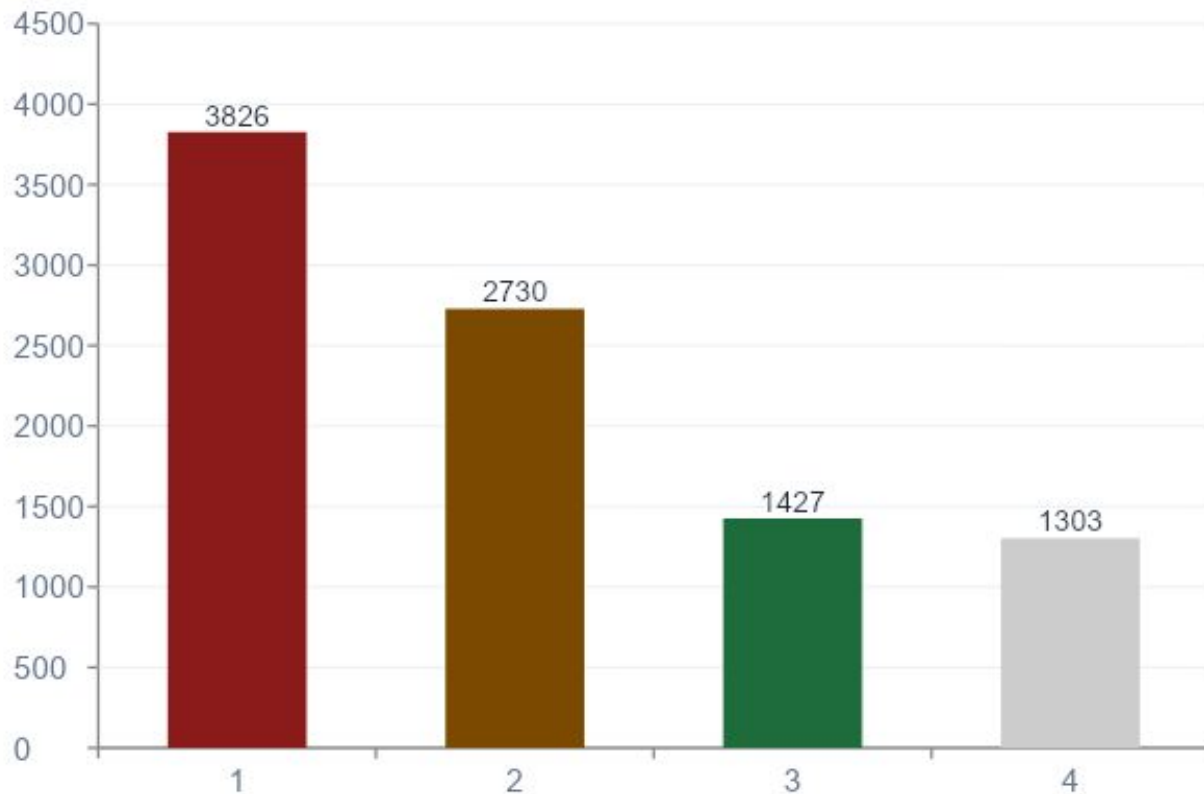
VERMONT HOSPITAL PERFORMANCE vs. NATIONAL BENCHMARKS (2023)

Measure	Vermont	Benchmark	Gap	Status
30-Day Readmission Rate	14.8%	14.5%	+0.3%	OK
Potentially Avoidable ED Visits	32.3%	No national benchmark yet	Major opportunity	ACTION
Operating Profit / Adj. Discharge (PPS)	\$5,345	\$4,162	+\$1,183 above benchmark	OK
Operating Profit / Adj. Discharge (CAH)	\$938	\$2,784	-\$1,846 BELOW benchmark	CRITICAL
Mgmt & Admin Cost / Adj. Discharge (PPS)	\$2,730	\$1,427	+\$1,303 excess (91% above)	CRITICAL
Vermonters with Personal Health Provider	91%	87% national avg	+4% above benchmark	STRONG
PCPs Accepting New Medicaid Patients	59%	Not set	Access gap for Medicaid patients	ACTION
ED Visits for MH/SIH per 10K ED visits	245.5	Not set nationally	High in Rutland, Windham counties	ACTION

VERMONT CIN: COE SPECIALTY CONCENTRATION BY HOSPITAL



THE ADMINISTRATIVE COST PREMIUM — VERMONT'S \$1,303 PER-DISCHARGE GAP



WHAT THIS MEANS

- Vermont PPS hospitals spend 91% MORE on administration than efficient peers
- This \$1,303/discharge gap = primary operational inefficiency target
- Shared services through Vermont CIN can close this gap
- Oliver Wyman: administrative reduction contributes >\$100M of \$400M+ transformation savings target
- AI scribes + credentialing automation + group purchasing = the operations playbook

THE RHRC METHODOLOGY — TECHNICAL ASSISTANCE FOR 14 HOSPITALS

Rural Health Redesign Center engagement — May through October 2025

1

Financial & Operational Baseline

Hospital-specific cost data from GMCB financial records and CMS cost reports. Position vs. Vermont average and national benchmark on key efficiency metrics.

2

Priority Identification

Working with hospital leadership to identify highest-leverage short-term cost reduction opportunities and medium-term transformation priorities.

3

Transformation Plan Development

Structured plans mapping specific actions to timelines, costs, and Act 167 outcome objectives. Hospital sustainability and cost analysis through service line review.

4

Regional Coordination

Connecting hospital-level planning to the statewide regional work. Ensuring individual plans are compatible with neighboring hospital plans. Identifying shared service opportunities.

CHAPTER 10

The Clinical Pillar — Redesigning Care Delivery

VERMONT BLUEPRINT FOR HEALTH — 15 YEARS OF RESULTS

\$5.8M

Reduction in medical expenditure

Per \$1M invested in Blueprint — documented 5.8:1 ROI (Population Health Management journal)

100%

Vermont PCMHs recognized by NCQA

All participating practices meet NCQA Patient-Centered Medical Home standards

77%

Hypertension controlled in PCMH patients

Blueprint quality metric; 4 points above national benchmark

91%

Vermonters with a personal provider

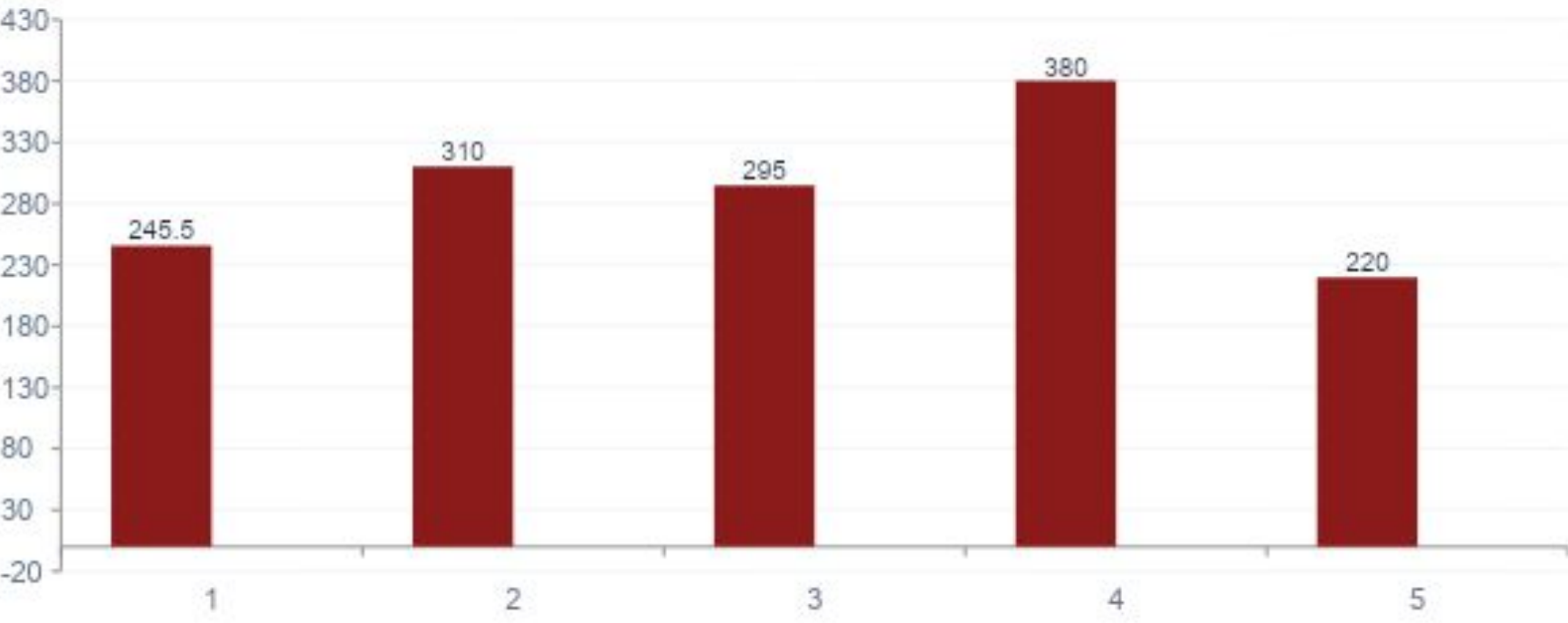
4 points above the 87% national benchmark — Blueprint's access foundation

COLLABORATIVE CARE MODEL (CoCM) — THE BEHAVIORAL HEALTH INTEGRATION STANDARD

The only integrated care model with 90+ randomized controlled trials and designated Medicare billing codes (CPT 99492, 99493, 99494)

Primary Care Provider (PCP)	Behavioral Health Care Manager (BHCM)	Psychiatric Consultant
<i>Treating & billing provider</i> Identifies patients via validated screening (PHQ-9, GAD-7). Provides warm handoffs. Bills CoCM codes monthly. Retains clinical ownership.	<i>Day-to-day care coordinator</i> Maintains patient registry. Provides brief interventions. Monitors treatment response with validated scales. Escalates to psychiatric consultant as needed.	<i>Clinical supervision</i> Regular case review with BHCM. Consultation on treatment recommendations. Direct care for highest-acuity cases only — one psychiatrist supports an entire practice.

VERMONT BEHAVIORAL HEALTH CRISIS: ED PRESENTATIONS BY COUNTY



245.5 ED visits per 10,000 for suicide ideation/self-harm — the clearest evidence of behavioral health access failure. 5 new CCBHCs by July 2026 and permanent CoCM funding through AHEAD EAST Fund are the operational response.

VERMONT'S THREE-LAYER BEHAVIORAL HEALTH ARCHITECTURE

Layer 1

Mental Health Integration in Primary Care

CoCM + Blueprint MHI pilot: behavioral health care managers embedded in PCMH practices. PHQ-9/GAD-7 screening. Brief intervention. 80% of Blueprint administrative entities increased CHT staffing in MHI pilot.

Funding vehicle: AHEAD EAST Fund (permanent MHI funding) + Blueprint PCMH

Layer 2

Certified Community-Based integrated Health Centers (CCBHCs)

Vermont CCBHC Demonstration State since 2024. 2 operational as of 2025. 5 new entities by July 2026 (RHT Program). Serve all patients regardless of insurance status. Comprehensive behavioral health including crisis intervention, SUD treatment, care coordination.

Funding vehicle: CCBHC Demonstration enhanced Medicaid + RHT Program grants

Layer 3

Crisis System and Acute Behavioral Health

Mobile crisis (operational January 2024). 988 Suicide and Crisis Lifeline centralized dispatch. 6 Designated Hospitals with inpatient psychiatric units. Brattleboro Retreat: primary psychiatric hospital. Hospital-level youth MH beds added under Act 167 actions.

Funding vehicle: State funding + Medicaid + Act 167 mandatory actions

CHAPTER 11

The AHS Restructuring Roadmap

THE FOUR AHS ORGANIZATIONAL GAPS OLIVER WYMAN IDENTIFIED

Gap 1: Program-Silo Structure

Problem: Organized around program types (Medicaid, mental health, SUD, aging), not geographic service areas. A hospital working on housing + MH + primary care must navigate 4+ AHS programs.

Fix: Designate HSA-level AHS Coordinators who convene all relevant programs around each hospital's transformation plan.

Gap 2: No Planning & Effectiveness Function

Problem: No analytical unit to model financial/access impact of service changes, evaluate hospital proposals, or monitor whether transformation investments are producing results.

Fix: Establish Division of Planning and Effectiveness. Joint AHS-GMCB analytics vendor (procurement underway 2025-2026).

Gap 3: Under-Resourced Transformation Mgmt

Problem: AHS must simultaneously manage AHEAD, 14 hospital plans, RHT Program (\$195M), GMCB coordination, and monthly legislative reporting. Insufficient staff for this portfolio.

Fix: Hire dedicated transformation staff using RHT Program funds (explicitly authorized). Establish Project Management Office.

Gap 4: Fragmented SDOH Integration

Problem: Housing, transportation, food assistance programs are separate from healthcare transformation goals. Cannot demonstrate SDOH → healthcare utilization reduction link.

Fix: Cross-program SDOH integration protocols. HSA Coordinator model links health and social services. VHCURES linkage to social service data.

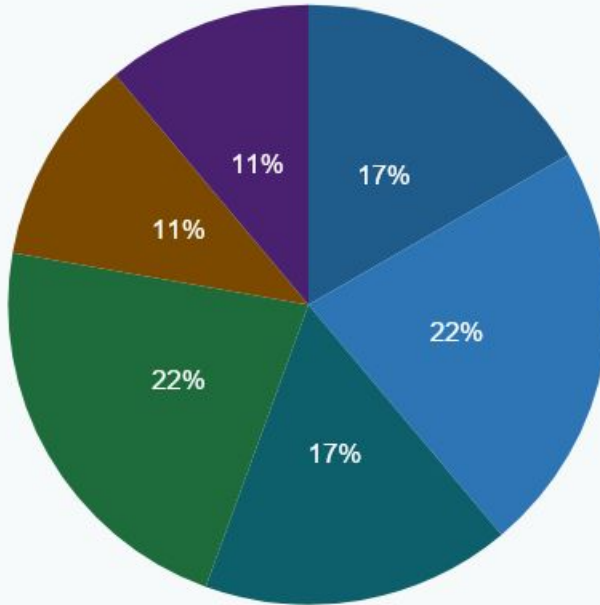
THE REDESIGNED AHS: THREE-LEVEL POPULATION HEALTH OPERATING MODEL

Level 1 STATEWIDE SYSTEM ARCHITECT	Strategic direction and six-pillar framework • Payment reform trajectory (with GMCB) • Capital allocation: RHT, EAST Fund, Act 68 grants Federal relationship management (CMS, HRSA, CMMI) • Accountability framework and monthly reporting • Statewide Strategic Plan development (Dec 2028)
Level 2 REGIONAL NETWORK CONVENER (14 HSAs)	HSA Coordinator for each of 14 Hospital Service Areas • Cross-program coordination: DVHA, DMH, Blueprint, ADS, DAIL • Hospital transformation plan integration and support Regional analytics using CIN analytics platform • Community engagement: hospitals, FQHCs, CBOs, SDOH programs
Level 3 PROGRAM DELIVERY OPERATOR	Medicaid (DVHA): AHEAD alignment, global budget Medicaid component • Blueprint for Health: PCMH expansion, MHI, CHT network • DMH: CCBHC development, crisis system, SUD continuum SDOH programs: cross-program integration via HSA Coordinator layer

VERMONT 2028 STATEWIDE STRATEGIC PLAN — PROPOSED STRUCTURE

Plan Section	Key Commitment Required
Executive Summary — Vermont's crisis, progress, central commitments	Quantified 2031/2035/2040 trajectory — specific, not aspirational
Section 1: System Context — Updated diagnosis as of 2028 baseline	Hospital financial positions updated with post-RBP/global budget data
Section 2: Payment Reform Architecture — RBP status, global budgets, AHEAD	RBP and global budget dollar savings documented by payer and HSA
Section 3: Primary Care & Behavioral Health — Blueprint, CCBHC, CoCM	AHEAD primary care investment % target vs. baseline
Section 4: Hospital Network & Regionalization — Tier designations, COE status	Confirmed tier designations — all 14 hospitals
Section 5: Data & Technology Infrastructure — VHCURES, HIE, analytics	VHCURES-VHIE integration status; statewide EHR decision
Section 6: Health Equity Strategy — Geographic, racial, SDOH equity	Disparity metrics by county — Northeast Kingdom as priority
Section 7: Workforce Strategy — Supply, deployment, retention	370 FTE gap plan with RHT workforce investment tracking
Section 8: AHS Organizational Architecture — HSA Coordinators, PMO	HSA Coordinator deployments confirmed; PMO operational
Section 9: Accountability Framework — 17+ Act 167/68 metrics	Performance targets for 2028-2031 on all 17+ measures
Section 10: Financial Plan — RHT balance, EAST Fund, projected savings	Capital allocation plan, projected system savings vs. Oliver Wyman baseline

HEALTH CARE DELIVERY ADVISORY COMMITTEE — ACT 68 MANDATE



■ State Government (AHS, GACB, HCA)
■ Hospitals & Health Centers
■ Health Insurers
■ Health Professions (MD, NP, RN, etc.)
■ Small Business
■ Long-Term Care

PRIMARY RESPONSIBILITIES:

- Set affordability benchmarks
- Monitor system performance
- Develop Statewide Strategic Plan (with AHS)
- Incorporate Comprehensive Primary Health Care Steering Committee recommendations
- Advise GACB on continuous evaluation process
- Provide coordinated recommendations to General Assembly
- Consult with Health Equity Advisory Commission

CHAPTER 12

The Equity Pillar — Closing Gaps

VERMONT'S GEOGRAPHIC EQUITY DISPARITIES — KEY COUNTY DATA

County / Region	Uninsured Rate	Key Disparity	Priority
Vermont statewide	~3%	Baseline	Reference
Essex County (NE Kingdom)	~8% — nearly 3× statewide	Highest uninsurance, most rural, highest poverty	CRITICAL
Orleans County (NE Kingdom)	6-7%	Limited provider access; North Country Hospital vulnerable	HIGH
Caledonia County (NE Kingdom)	5-6%	NE VT Regional Hospital serves area; financial fragility	HIGH
Rutland County	~3.5%	Higher opioid overdose rates; above-avg MH crisis ED presentations	ELEVATED
Windham County	~4%	Higher suicide/SH and opioid rates; SUD corridor with Springfield Hospital	ELEVATED
Chittenden County (Burlington)	~2.5%	Best-served county; UVMMC anchor	BASELINE
Black adults (statewide)	~18% uninsured (vs. 6% white adults)	82% have insurance vs. 94% statewide; 80% have personal provider vs. 91%	SIGNIFICANT

HEALTH DISPARITY ROOT CAUSE TAXONOMY — MATCHING INTERVENTIONS TO CAUSES

1

Geographic Access Barriers

MECHANISMS:

Distance; transportation; provider shortage; broadband gaps; limited hours

INTERVENTIONS:

COE regionalization + EMS; telehealth (RHT); broadband investment; Blueprint expansion

2

Social Needs Barriers

MECHANISMS:

Housing instability; food insecurity; transportation; economic stress

INTERVENTIONS:

AHEAD EAST Fund SDOH; Blueprint HRSN screening; AHS HSA Coordinators; CHW deployment

3

Clinical Practice Variation

MECHANISMS:

Weight bias; racial bias in treatment; cultural incompetence; language barriers

INTERVENTIONS:

Provider diversity recruitment; implicit bias training; standardized protocols; patient advisory councils

4

Coverage & Cost Barriers

MECHANISMS:

High deductibles; copays causing care avoidance; coverage gaps (GLP-1s for obesity)

INTERVENTIONS:

Act 68 affordability benchmarks; RBP premium passthrough; BALANCE Model GLP-1 decision

5

Trust & Engagement Barriers

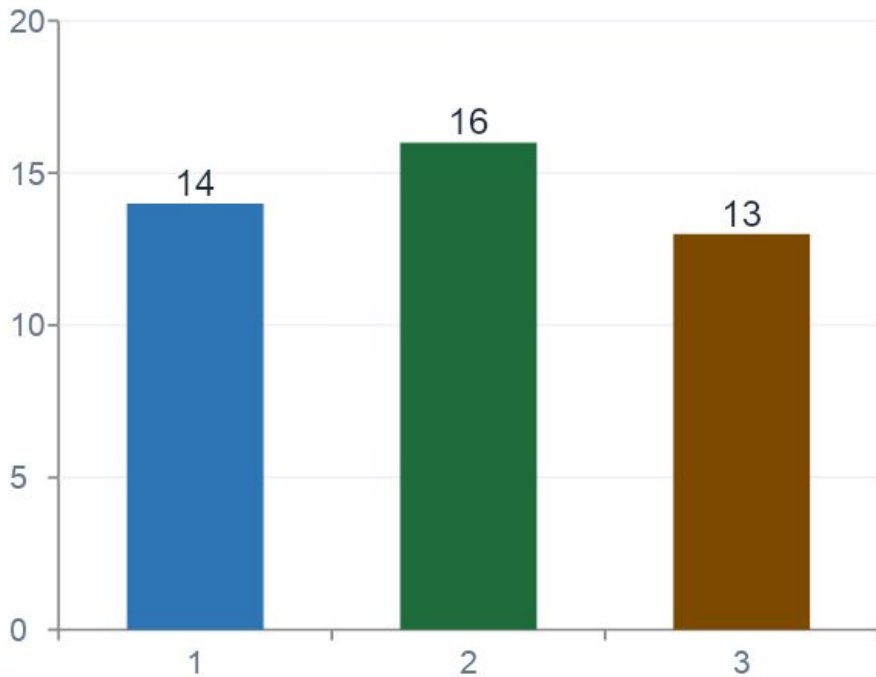
MECHANISMS:

Discrimination in care; stigma; historical mistreatment; lack of culturally competent providers

INTERVENTIONS:

CHWs from served communities; HEART program (LGBTQ+); BIPOC community partnerships; CCBHCs

THE GLP-1 ACCESS EQUITY CRISIS — WHO GETS TRANSFORMATIVE OBESITY DRUGS



CMS BALANCE Model (Dec 2025)

Voluntary model for Medicaid agencies to cover GLP-1s for obesity at negotiated prices. Launch: May 2026. Vermont DVHA must opt in.

Drug Costs After Negotiation

IRA negotiated price: \$274/month (effective 2027) vs. \$1,000+ list price. Still out of reach without Medicaid coverage for Vermont's lowest-income patients.

Vermont's Equity Decision

Vermont Medicaid covers GLP-1s for diabetes but NOT obesity. BALANCE opt-in is an equity policy choice: Medicaid patients have higher obesity, diabetes, and cardiovascular disease rates.

VERMONT EQUITY ACCOUNTABILITY FRAMEWORK — 2028

STRATEGIC PLAN TARGETS

Equity Indicator	Stratification	Current Status	Target Direction
Uninsured rate	By county (focus: Essex, Orleans, Caledonia)	Essex: ~8% vs. 3% statewide	Reduce NE Kingdom gap toward statewide 3% by 2030
Primary care access rate	By race/ethnicity (BIPOC vs. white)	BIPOC adults: 79-81% vs. 91% statewide	Reduce gap — target 87%+ for all race/ethnicity groups
Potentially avoidable ED visits	By county; by income quintile	32.3% statewide — higher in rural counties	Track NE Kingdom rate; reduce toward statewide average
30-day MH/SUD follow-up	By county; by race/ethnicity	76% MH / 68% SUD — gaps in Rutland, Windham	Increase toward 85%+ in all counties
GLP-1 access for Medicaid enrollees	By income; by county	Not covered for obesity indication	BALANCE Model opt-in decision 2026
SDOH screening completion	By HSA; by patient demographics	Operational in Blueprint PCMHs; not uniform	Universal completion in all Blueprint PCMHs by 2027
Emergency response time	By county (focus: NE Kingdom)	No current baseline	Establish baseline; track improvement via EMS regionalization

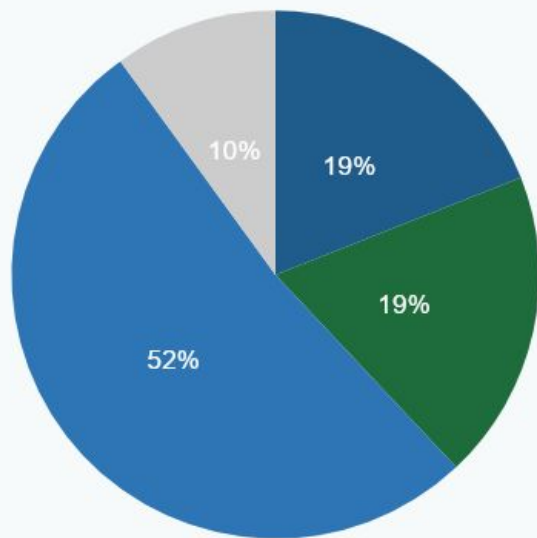
CHAPTER 14

The Technology Pillar — Data Infrastructure

VERMONT'S TARGET TECHNOLOGY ARCHITECTURE — FIVE LAYERS

Layer 5	Advanced Analytics & AI Applications Population health analytics platform (AHS-GMCB) · AI scribe grants (RHT) · Diagnostic AI (CIN) · RPM · Price transparency
Layer 4	Population Health Management Blueprint clinical data registry · Attribution management · SDOH screening data · CHT care coordination registry
Layer 3	Integrated Health Record (Target State) VHCURES-VHIE integration · MH and SUD data (42 CFR Part 2 compliant) · SDOH data · Single patient identity
Layer 2	Data Exchange Infrastructure Vermont VHIE/VITL · FHIR R4 APIs · Vermont CIN data standards · CommonWell/Carequality national connectivity
Layer 1	Core Data Assets (Foundation) VHCURES (100% Medicare/Medicaid; 60% commercial) · VUHDDS (14 hospitals) · 4 EHR platforms · Blueprint clinical registry · GMCB regulatory data

VHCURES — VERMONT'S ALL-PAYER CLAIMS DATABASE: COVERAGE AND GAPS



- Medicare (100% of enrollees)
- Medicaid (100% of enrollees)
- Commercial (~60% — ERISA gap)
- Not in VHCURES (~10% of state)

ERISA Gap (~40% of commercial)

Self-insured employers cannot be required by states to submit data (federal law). Many large Vermont employers excluded.

Impact: Total cost of care calculations systematically undercount commercially insured population.

12-18 Month Data Lag

Most VHCURES data reflects conditions 12-18 months prior. November 2025 AHS report acknowledged most measures reflect 2023 or earlier.

Impact: Cannot manage global budgets in real-time. AHS-GMCB analytics vendor procurement addresses this.

No Behavioral Health / SDOH Data

MH/SUD records governed by 42 CFR Part 2. Social service utilization not in VHCURES.

Impact: Cannot link housing/transportation/food programs to healthcare outcomes — breaks SDOH → utilization analysis.

VERMONT'S THREE PRIORITY FHIR USE CASES — FROM STANDARD TO WORKING SYSTEM

1

Patient Access API — §170.315(g)(10) compliance

Enables: Patients access their health records through third-party apps. Required by CMS for hospitals and clinicians receiving Medicare/Medicaid payments.

Vermont: All Vermont hospitals must be compliant. Monitor smaller CAHs without robust IT departments.

2

Provider Access API — Care coordination across CIN

Enables: Care coordinators at one organization can query clinical records from another for shared patients. Enables real-time cross-facility care coordination.

Vermont: Priority RHT IT investment. Enables inter-hospital care coordination that regionalization blueprint requires.

3

Payer-to-Provider API — Electronic prior authorization

Enables: FHIR-based prior authorization eliminates manual PA. Formulary exchange reduces prescription errors. Uses Da Vinci CRD and DTR implementation guides.

Vermont: Accelerates Vermont's PA reform agenda. GMCB should require payer FHIR API compliance as condition of rate approval.

AI SCRIBE TECHNOLOGY — WORKFORCE MULTIPLIER FOR VERMONT'S RURAL PRACTICES

2+ hrs

Daily documentation time saved per provider

AI scribe automates transcription of clinical encounters

370 FTE

Vermont primary care shortage by 2030

AI scribe = equivalent capacity gain without hiring

>25%

Effective capacity gain per provider

At 8 patients/day, 2 hours saved = 4+ additional appointments

\$195M

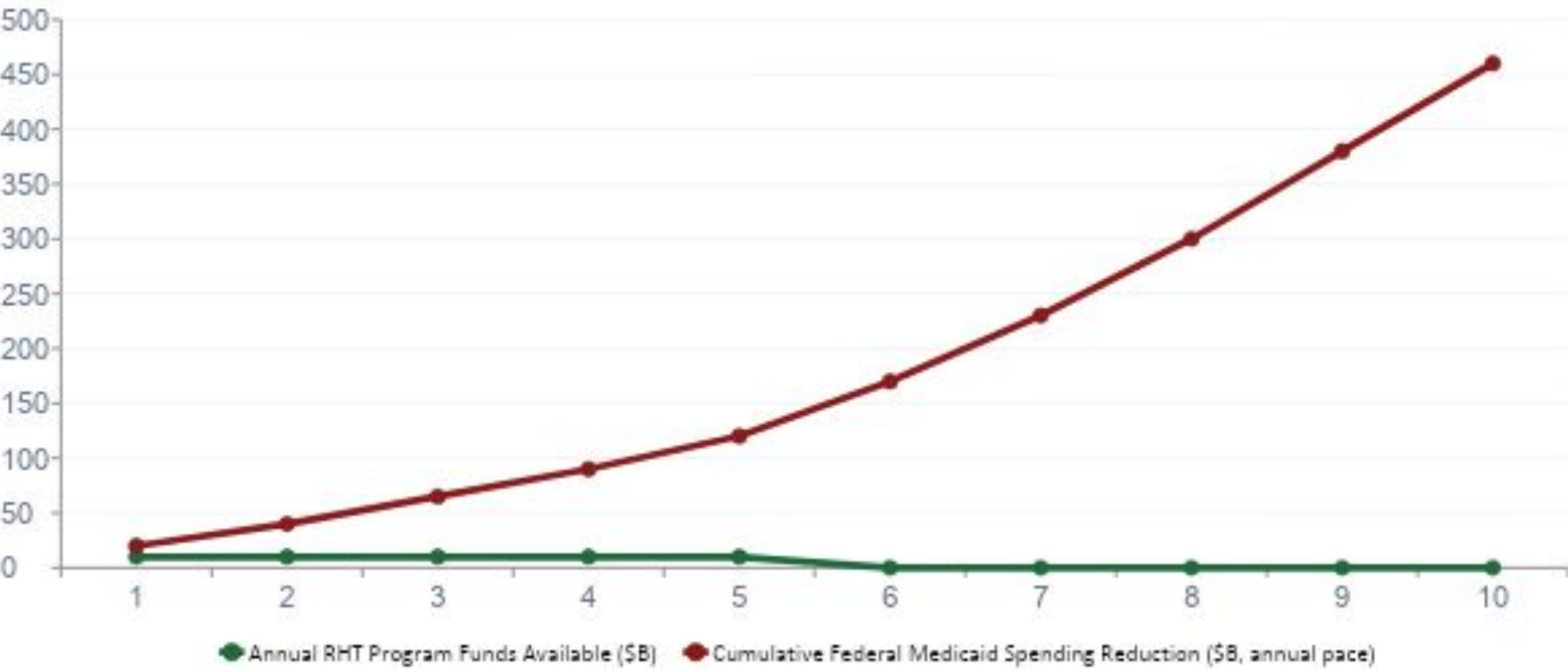
RHT Program — includes AI scribe grants

Explicitly for rural/independent practices that can't afford enterprise pricing

CHAPTER 17

The Future — 2026 and Beyond

THE RURAL HEALTH FISCAL CLIFF: RHT FUNDS vs. MEDICAID CUT TIMELINE



64% of Medicaid cuts hit AFTER 2030 when RHT funds expire. Vermont must achieve structural efficiency by 2030 to survive the back-loaded cuts.

VERMONT 2026–2030: TWO SCENARIOS

TRANSFORMATION SUCCESS SCENARIO

FY2027: RBP implemented. First premium growth slowdown in a decade.

FY2028: Non-CAH global budgets operational. AHEAD delivers population health improvements.

2028: AHS delivers Strategic Plan with specific quantified commitments.

2030: RHT capital fully deployed. Medicaid cuts absorbed through efficiency gains.

2031-35: Vermont as national template. AHEAD expands nationally using Vermont evidence.

PARTIAL TRANSFORMATION SCENARIO

FY2027: RBP contested — UVMMC seeks legislative relief. Modified glide path.

FY2028: Global budget methodology not finalized. Deadline extended.

2028: Strategic Plan descriptive, not prescriptive — identifies questions, avoids commitments.

2030: RHT capital partially for operations support, not structural transformation.

2031-35: Transformation real but incomplete. Second reform wave required.

SIX-PILLAR FORECAST 2026–2035: HEADWINDS AND TAILWINDS

Pillar	Primary Headwind	Primary Tailwind	2035 Outlook
Policy	H.R.1 Medicaid cuts; federal policy instability; work requirements	AHEAD expanding; mandatory RBP/global budgets; price transparency momentum	Moderate national progress; Vermont high if execution holds
Economics	Medicaid revenue reductions; GLP-1 spending wave	RBP producing savings; IRA drug negotiation; ACO REACH maturing	VBC reaches 70-75% commercial by 2030; global budgets beyond Vermont/MD limited
Technology	Data lag; AI governance absent; FHIR incomplete at smaller facilities	CIN analytics; RHT IT grants; ambient AI reducing burden; FHIR maturing	Fastest-improving pillar nationally; AI governance the critical risk
Clinical	Primary care workforce shortage; MHI sustainability; PACE gap	Blueprint + AHEAD primary care investment; CCBHC expansion; CoCM billing	Steady improvement nationally; Vermont has country's strongest primary care infrastructure
Equity	H.R.1 coverage losses hitting most vulnerable; federal equity funding cuts	BALANCE GLP-1; CCBHC access policy; HRSN screening; Act 68 equity requirements	Highest-risk pillar nationally; explicit equity targets in Vermont Strategic Plan essential
Operations	Hospital transformation pace; AHS capacity gaps; administrative complexity	\$195M RHT; all 14 hospitals engaged; GMCB capacity added; CIN shared services	Vermont: transformation underway; question is whether execution matches deadlines

VERMONT IS NOT JUST VERMONT.

*It is every state,
ten to twenty years from now.*

The forces producing Vermont's crisis — demographic aging, hospital financial fragility, commercial cross-subsidy collapse, voluntary reform failure — are present nationally.